

HOSPITAL INSOLVENCY UNDER THE IBC: CLINICAL MORATORIUM, PATIENT-DATA TRUSTS, CBDC MICRO-TRANCHES

Introduction

When a normal business collapses, delay burns cash; when a hospital fails, delay harms people. India's Insolvency and Bankruptcy Code (IBC) bring order to corporate distress through the Corporate Insolvency Resolution Process (CIRP), but the rules were written with factories in mind rather than intensive care units. A legal moratorium can pause lawsuits and calls from creditors.¹ It cannot keep oxygen flowing at night, reassure clinicians that surgeries will start on time, or help bidders judge clinical risk without mishandling patient records. This essay sets out a hospital-specific plan that fits within the IBC. It proposes three linked steps: a Clinical Moratorium to keep emergency care, intensive care, operating theatres, oxygen, blood banks, dialysis, and diagnostics running while the statutory clock continues;² Patient-Data Trusts so bidders can evaluate performance using consented, limited data; and a Central Bank Digital Currency (CBDC) based micro-tranches that pay critical suppliers as soon as a service is verified. The aim is simple: protect patients, preserve value, and keep the case on time.

Why Hospitals Require a Sector-Specific Insolvency Playbook

A hospital keeps its value only when care is safe and uninterrupted. Even a short disruption in the emergency department, intensive care and operating rooms, oxygen deliveries, blood banking, dialysis services, or core diagnostics can cancel surgeries, push staff to leave, and lower bids. The IBC already requires the business to keep running³ and protects essential supplies during the moratorium.⁴ However, it defines “essentials” as utilities such as electricity, water, telecom, and IT rather than clinical services.⁵ This essay translates those general rules into clear, hospital-specific steps that a resolution professional (RP) can apply from day one, so patient safety and financial value rest on the same foundation.

¹ The Insolvency and Bankruptcy Code, 2016 (31 of 2016) s 14(1).

² The Insolvency and Bankruptcy Code, 2016 (31 of 2016) s 12.

³ The Insolvency and Bankruptcy Code, 2016 (31 of 2016) s 20.

⁴ The Insolvency and Bankruptcy Code, 2016 (31 of 2016) s 14(2).

⁵ Insolvency and Bankruptcy Board of India (Insolvency Resolution Process for Corporate Persons) Regulations, 2016 reg 32.

Pillar I: Statutory Clinical Moratorium within the Existing CIRP Timeline

A Clinical Moratorium is a practical rule rather than a loophole or a delay. When CIRP begins, the RP names the hospital's critical services, namely emergency care, intensive care, operating theatres, oxygen and vacuum lines, the blood bank, dialysis, and core diagnostics such as imaging and laboratory services, and protects these services as strictly as power and water. The timelines do not change because the case still runs within the IBC limits. The moratorium simply defines what continuity means in a hospital so that direct patient care is not delayed by legal processes.⁶

Payments stay strict and predictable. Pay for current use now and route older dues through the resolution plan. Vendors are paid once the delivery is verified. A short grace window, measured in hours, prevents paperwork from creating clinical risk, while preserving the vendor's right to stop new deliveries if current bills are not paid.⁷

Accountability focuses on results rather than paperwork. If anyone asks the court for more time, that request should be earned by showing that continuity actually improved. Use three simple checks. First, were beds and other critical services available when patients needed them? Second, did cancellations of surgeries and other time-sensitive procedures go down? Third, did expected recovery for creditors improve compared with liquidation value because care continued safely? These checks keep the focus on what matters: direct patient care and value for stakeholders.⁸

Pillar II: Patient-Data Trusts for Privacy-Preserving Diligence

Bidders cannot price a hospital from revenue and cost lines alone. They also need a clear picture of the care being delivered, who is treated, what outcomes are achieved, and what kind of risk exist. Most of this information present inside electronic medical records and related clinical systems. Because these records are sensitive and governed by consent-based law, a neutral governance layer is required.⁹

⁶ The Insolvency and Bankruptcy Code, 2016 (31 of 2016) ss 12, 20.

⁷ The Insolvency and Bankruptcy Code, 2016 (31 of 2016) s 14(2A).

⁸ *Committee of Creditors of Essar Steel India Ltd v Satish Kumar Gupta* (2020) 8 SCC 531.

⁹ Digital Personal Data Protection Act, 2023 (22 of 2023) ss 6(1), 6(4), 6(7)– (9).

A Patient-Data Trust provides that layer without creating a new database. The hospital keeps the records, and the trust coordinates lawful access. Using existing health-data consent mechanisms,¹⁰ the trust grants time-limited and purpose-limited access to only the fields required for diligence, such as case-mix and payer-mix profiles, infection rates, length-of-stay patterns, equipment uptime, and discharge outcomes. Identities are not shared. Every access is logged from start to finish, and consent can be withdrawn.¹¹ In practice, bidders see the “what” of clinical performance while the “who” remains protected. This reduces privacy risk, increases bidder confidence, and supports stronger pricing without compromising rights.

Pillar III: CBDC Micro-Tranches for Just-in-Time Funding of Clinical Essentials

Hospitals run on minute-by-minute logistics. If an oxygen refill is acknowledged at 6:10 p.m., the supplier should not wait days to be paid. Micro-tranches close this timing gap by putting a small programmable escrow inside the resolution plan. If a digital-rupee (CBDC) wallet is available, it serves as the payment rail;¹² if not, a tightly specified bank escrow with API-based releases serves the same role. Each clinically important event for example an oxygen cylinder signed in on the ward, pharmacy stock rising above a safety threshold, or an implant staged in an operating theatre which triggers a small, pre-set payment from escrow. Weekly caps, automated alerts, and clawbacks handle disputes and discourage misuse. In short, the rule “pay current dues to keep essentials flowing” happens in near real time, within minutes rather than after a committee meeting.¹³ This keeps direct patient care stable and preserves the hospital’s going-concern value.

Synthesis and Implementation: Doctrinal Fit within the IBC Framework

All three elements sit within powers the IBC already gives RPs and courts. A resolution plan must show how operations will continue, how stakeholders are protected, and how performance will be

¹⁰ National Institution for Transforming India, ‘National Digital Health Mission: Strategy Overview’ (2020) https://www.niti.gov.in/sites/default/files/2023-02/ndhm_strategy_overview.pdf accessed 8 October 2025.

¹¹ National Health Authority, Ministry of Health and Family Welfare, ‘ABDM: Health Information Exchange and Consent Manager (HIE-CM) — consent-based sharing’ (Press material, 14 October 2024) <https://www.mohfw.gov.in/?q=en/pressrelease-87> accessed 10 October 2025.

¹² Reserve Bank of India, ‘Digital Rupee (e₹) – FAQs’ (9 January 2025) <https://www.rbi.org.in/commonman/English/Scripts/FAQs.aspx> accessed 11 October 2025.

¹³ Reuters, ‘India to broaden e-rupee’s retail access beyond banks, cenbank says’ (5 April 2024) <https://www.reuters.com/markets/currencies/india-broaden-e-rupee-distribution-beyond-banks-boost-retail-use-cenbank-says-2024-04-05/> accessed 12 October 2025.

supervised.¹⁴ The Clinical Moratorium turns those general duties into concrete clinical obligations and assigns responsibility for keeping them in place. The Data-Trust clause codifies lawful, audited, limited-purpose diligence access without bulk copying or open-ended permissions. The Digital Escrow clause defines eligible vendors and the verification events that trigger micro-payments, with caps and clawbacks built in. An outcome-linked extension clause ties any extra time to measurable gains in continuity and expected recovery, so the statutory clock is respected and extensions buy real public value rather than breathing space alone.

Anticipated Objections and Structured Responses

Privacy worries are addressed by design. The Patient-Data Trust does not centralize records; it uses explicit consent, narrows access to what is strictly necessary for a short window, logs every read, and allows revocation. Questions about CBDC readiness are handled by being rail-agnostic: where digital-rupee wallets are practical, they are used; where they are not, a programmable bank escrow achieves the same operational effect. Fears that these mechanisms might slow the case miss the point. The Clinical Moratorium keeps the matter on the statutory clock by removing hesitation at the point of care; micro-payments prevent standoffs with essential vendors that would otherwise force adjournments; and any extension must be justified with hard numbers rather than hopeful assurances. Once running, clinicians know supplies will arrive and be paid for, bidders know what they are buying, and courts see objective indicators rather than ambiguity. Finally, emergency-care jurisprudence¹⁵ and national standards under the Clinical Establishments Act¹⁶ reinforce the principle that critical care must not be interrupted, which aligns with the Clinical Moratorium's focus on safety during CIRP.

Conclusion

The IBC promises speed, order, and outcomes better than liquidation. In healthcare, that promise is inseparable from patient safety. A Clinical Moratorium stabilizes the services that define a hospital's value; Patient-Data Trusts allow bidders to understand performance without exposing identities; and CBDC-style micro-tranches, or their escrow equivalent, move money at the speed

¹⁴ IBC ss 12, 14, 20; IBBI (CIRP) Regulations 2016 reg 32.

¹⁵ *Pt Parmanand Katara v Union of India* (1989) 4 SCC 286.

¹⁶ Clinical Establishments (Registration and Regulation) Act, 2010 (23 of 2010).

of care. Together, these elements turn the usual tension between speed, value, and protection into a self-reinforcing triangle. Because each piece fits within the tools the law already provides, this is a practical path to hospital resolutions that are fast, valuable, and safe by 2030.